
COLD ISCHEMIA FOUNDATION

Ellenton, Florida | Congressional District FL-16 | coldischemia.foundation

FORMAL CORRESPONDENCE — CONFIDENTIAL

March 30, 2026

TO:

The Honorable Ben Albritton, President
Florida State Senate
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404 South Monroe Street
Tallahassee, Florida 32399-1100
Tel: (850) 487-5229

The Honorable Daniel Perez, Speaker
Florida House of Representatives
Suite 420, The Capitol
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RE:

FORMAL DEMAND FOR STATE LEGISLATIVE ACTION — CARE PARTNER COMPENSATION, RESPITE SERVICES, AND WORKFORCE PROTECTION UNDER FLORIDA MEDICAID

To the Presiding Officers of the Florida Legislature:

This correspondence is submitted not as a request for consideration, but as a formal demand for corrective legislative and regulatory action. The systemic failure of federal and state government to recognize, protect, and compensate unpaid family care partners — across every chronic disease category — constitutes one of the most consequential policy failures of the last three decades. That failure is well-documented, empirically measurable, and morally indefensible. It will not continue without challenge.

I write to you as a three-time kidney transplant recipient, a Lean Six Sigma analyst, and a citizen who has witnessed firsthand the systematic abandonment of the individuals who sustain our healthcare system at no cost to the federal treasury. The care partner workforce — estimated at 53 million Americans providing unpaid labor valued at approximately \$600 billion annually (Reinhard et al., AARP Public Policy Institute, 2023) — is invisible to every budget appropriation, every CMS reimbursement schedule, and every legislative session convened in this country. That invisibility is not an accident. It is a policy choice. And it is a choice your office has made, repeatedly, by inaction.

As the presiding officers of the Florida Legislature representing Congressional District FL-16 and the broader State of Florida, you hold direct authority over the state Medicaid framework, the appropriations process, and the statutory infrastructure that could — and must — be reformed to address the documented care partner crisis. Florida has enacted no meaningful statutory framework

requiring compensation, workforce protections, or standardized respite services for care partners across chronic illness categories. That gap is a policy choice this Legislature has made. It is a policy choice this Legislature must reverse.

I. THE SCOPE OF GOVERNMENTAL FAILURE

The United States government has long acknowledged the existence of the care partner crisis while simultaneously refusing to act on it in any substantive, enforceable manner. The National Academies of Sciences, Engineering, and Medicine (NASEM) issued its landmark report, *Families Caring for an Aging America* (NASEM, 2016), nearly a decade ago. That report described family caregiving as a "public health issue" with severe consequences for caregiver health, financial security, and workforce participation. Congress received those findings. Congress did nothing.

The Government Accountability Office has documented, on multiple occasions, the failure of federal agencies to coordinate care partner support services across disease programs (GAO-18-88, January 2018). The Office of Inspector General has identified gaps in beneficiary protections that disproportionately fall on family members providing informal care (OIG Report OEI-02-17-00200, 2018). The Senate Special Committee on Aging has held hearings. Reports have been published. Recommendations have been issued. Zero binding, enforceable, cross-disease compensation mandates exist at the federal level. Not one.

At the state level, Florida has enacted no meaningful statutory framework requiring compensation, workforce protections, or standardized respite services for care partners across chronic illness categories. The state's Medicaid waiver programs offer fragmented, disease-specific, and means-tested relief that leaves the vast majority of care partners without any financial recognition whatsoever. This is unacceptable.

II. THE DOCUMENTED HUMAN AND ECONOMIC COST

The consequences of legislative inaction are not abstract. They are quantifiable, published, and have been placed before this body repeatedly without result.

Approximately 61% of care partners are employed while providing care, and 69% of those individuals report making significant workplace adjustments — including reduced hours, declined promotions, or full workforce departure — as a direct result of caregiving responsibilities (National Alliance for Caregiving & AARP, *Caregiving in the U.S.*, 2020). This represents a structural suppression of labor force participation with cascading effects on Social Security eligibility, retirement savings, and lifetime earnings — costs that are ultimately borne by the federal government in the form of increased entitlement dependency later in life.

Care partners report dramatically higher rates of depression, anxiety, hypertension, and immune dysfunction than non-caregiving peers (Schulz & Sherwood, *American Journal of Geriatric Psychiatry*, 2008). A 2022 analysis published in *JAMA Internal Medicine* found that intensive caregiving is independently associated with a 23% increased risk of all-cause mortality among care partners themselves (Roth et al., *JAMA Internal Medicine*, 2022). This government is, by policy, presiding over the slow deterioration of the workforce that keeps its most vulnerable citizens alive. That is not a

rhetorical claim. It is a peer-reviewed finding.

The economic calculus is equally stark. The Medicaid program spent \$613 billion in fiscal year 2022 (CMS National Health Expenditure Data, 2023). Informal care partners are estimated to offset formal care costs by a ratio of nearly 3:1 in community-based settings (Feinberg & Levine, *Health Affairs*, 2015). Every dollar not invested in care partner support is a dollar that will eventually be demanded from the formal care system — at substantially greater cost to taxpayers. The refusal to establish compensation mandates is not fiscal conservatism. It is deferred liability.

III. THE SPECIFIC FAILURE OF DISEASE-SILOED POLICY

Federal and state care partner policy, to the extent it exists at all, is organized around disease categories rather than around the functional reality of caregiving. Programs under the National Family Caregiver Support Program (NFCSP), authorized under the Older Americans Act, are age-gated and disease-agnostic in theory but administratively fragmented in practice. Medicaid Home and Community-Based Services (HCBS) waivers are state-administered, inconsistently funded, and routinely under-resourced. No federal statute mandates a universal, cross-disease compensation floor for informal care partners. None.

A care partner managing end-stage renal disease has no more federal protection than a care partner managing ALS, Alzheimer's, multiple sclerosis, or heart failure. The disease differs. The policy vacuum does not. This reflects a structural design failure, not a resource limitation. Congress has the authority and the appropriations infrastructure to correct this. It has chosen not to exercise that authority.

IV. WHAT IS REQUIRED — IMMEDIATELY This letter does not ask. It identifies the minimum corrective actions that the recipient of this correspondence is required to undertake as a function of public office and the obligation to the constituents this office represents. The following are not optional recommendations. They are the floor of an adequate governmental response to a documented, quantified, and long-ignored crisis.

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1. STATUTORY CARE PARTNER COMPENSATION PROGRAM

The Florida Legislature must establish a statutory care partner compensation program within the existing Medicaid framework, extending eligibility across all chronic disease categories without disease-category restriction. Florida's current Medicaid waiver programs leave the vast majority of care partners without any financial recognition. This must be corrected in the next legislative session.

2. RESPITE CARE APPROPRIATION

The Legislature must appropriate dedicated funding for structured respite care services available to all qualifying care partners regardless of the care recipient's specific diagnosis. Respite care reduces care partner burnout, reduces care recipient hospitalization, and produces net fiscal savings to the Medicaid program. The failure to fund it is a policy choice with documented costs.

3. WORKFORCE RE-ENTRY ASSISTANCE

The Legislature must establish a care partner workforce re-entry assistance program providing employment support, retraining, and bridge benefits for individuals who have exited the labor force to provide informal care. The suppression of care partner workforce participation has cascading effects on Social Security eligibility, retirement savings, and lifetime earnings — costs ultimately borne by state and federal programs.

4. MENTAL HEALTH SERVICES FOR CARE PARTNERS

The Legislature must require that Florida's Medicaid program cover mental health services for qualifying care partners. The documented rates of depression, anxiety, and burnout among care partners — and the clinical consequences for the recipients they support — provide a clear actuarial basis for this coverage expansion.

5. FORMAL RESPONSE REQUIRED

Your office is required to provide a written response within thirty (30) calendar days detailing the specific legislative steps it will take in response to the demands herein. Failure to respond will be treated as a matter of public record and disclosed accordingly.

V. CLOSING STATEMENT

The individuals who provide unpaid care to chronically ill, disabled, and dying Americans are not a special interest. They are a structural pillar of the United States healthcare system — one your office has benefited from financially while refusing to acknowledge legislatively. That arrangement is over.

I am a transplant recipient who is alive because someone cared for me without compensation, without recognition, and without the support of a single federal or state program designed for her benefit. That person's labor — and the labor of 53 million Americans like her — will no longer be treated as a free resource available for governmental exploitation. The data is clear. The cost is documented. The authority to act exists. What has been absent is the political will. This letter demands its exercise.

You were elected or appointed to serve the public interest. The care partner population is the public interest. Act accordingly.

Respectfully and without reservation,

Jeff Parke

Co-Founder & Executive Director

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CC:

The Honorable Ron DeSantis, Governor, State of Florida

The Honorable Robert F. Kennedy, Jr., Secretary, U.S. Department of Health and Human Services

Dr. Mehmet Oz, Administrator, Centers for Medicare & Medicaid Services

Sources cited herein include: Reinhard et al., AARP Public Policy Institute (2023); National Academies of Sciences, Engineering & Medicine (NASEM, 2016); GAO Report GAO-18-88 (January 2018); OIG Report OEI-02-17-00200 (2018); National Alliance for Caregiving & AARP, Caregiving in the U.S. (2020); Schulz & Sherwood, American Journal of Geriatric Psychiatry (2008); Roth et al., JAMA Internal Medicine (2022); CMS National Health Expenditure Data (2023); Feinberg & Levine, Health Affairs (2015).